

Arthritis Fatigue: Why You're So Tired and How to Manage It

Understanding and managing arthritis fatigue. Discover why arthritis makes you exhausted, evidence-based strategies for energy conservation, pacing and better sleep.

By **Living With Arthritis Editorial Team** · Reviewed by **Clinical Advisory Panel** (HCPC-registered physiotherapy & rheumatology nursing input) · 2026-01-30 · livingwitharthritis.org.uk

If you live with arthritis and feel persistently exhausted — far beyond normal tiredness — you are not imagining it and you are not alone. Fatigue affects up to **80% of people** with inflammatory arthritis and is frequently cited as one of the most disabling symptoms, sometimes even more so than pain. Unlike ordinary tiredness, arthritis fatigue is a deep, overwhelming exhaustion that doesn't fully resolve with rest. This guide explains why arthritis causes fatigue and provides practical, evidence-based strategies for managing it.

Why Does Arthritis Cause Fatigue?

Arthritis fatigue is multifactorial — it results from the interaction of several mechanisms:

Inflammation

In inflammatory types of arthritis (RA, PsA, AS), the immune system produces pro-inflammatory cytokines (TNF- α , IL-1, IL-6) that cause systemic effects throughout the body. These chemical messengers don't just affect joints — they act on the brain and nervous system to produce fatigue, similar to the exhaustion you feel during flu. This is why fatigue is often worst during flare-ups and improves when disease activity is controlled.

Pain

Chronic pain is physically and mentally exhausting. The nervous system works overtime processing pain signals, leaving fewer resources for other activities. Pain disrupts sleep

quality, creating a fatigue cycle. Pain-related inactivity leads to deconditioning, which increases fatigue with even minor exertion.

Sleep Disruption

Up to 70% of arthritis patients report poor sleep. Night pain wakes you or prevents deep sleep. Stiffness and discomfort make it hard to find a comfortable position. Medications (particularly corticosteroids) can disrupt sleep. The anxiety and depression associated with chronic pain further impair sleep quality.

Anaemia

"Anaemia of chronic disease" occurs in many people with inflammatory arthritis. Chronic inflammation suppresses red blood cell production, leading to mild anaemia that contributes to fatigue. Your GP can check for this with a simple blood test.

Medications

Several arthritis medications can cause fatigue as a side effect, including methotrexate (particularly in the 24–48 hours after taking it), sulfasalazine, pain medications (particularly opioids and codeine), and some biologics. If you suspect your medication is contributing to fatigue, discuss this with your rheumatologist — alternatives may be available.

Emotional Factors

Depression and anxiety are common in arthritis and significantly worsen fatigue. The frustration of living with limitations, grief for lost abilities and worry about the future all drain emotional energy. Addressing mental health is a crucial part of fatigue management.

Evidence-Based Fatigue Management Strategies



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1. Pacing

Pacing is the most consistently recommended strategy for arthritis fatigue. It means balancing activity and rest throughout the day to avoid the "boom and bust" pattern (overdoing it on good days, then crashing for days afterwards). Break tasks into smaller chunks with rest periods between. Plan your most demanding activities for when your energy is highest (usually mid-morning). Alternate between physically demanding and lighter activities. Use a diary to identify your energy patterns and plan accordingly. It's not about doing less — it's about doing things more evenly.

2. Exercise

Counterintuitive as it seems, regular exercise **reduces** fatigue. A Cochrane review found that aerobic exercise significantly improved fatigue in people with RA. Start with just 5–10 minutes of gentle walking or swimming, then build up gradually. Consistency is more important than

intensity. Aim for some form of movement every day, even on bad days. Exercise improves sleep quality, mood and physical conditioning — all of which combat fatigue.

3. Sleep Hygiene

- Maintain a consistent sleep schedule (same bedtime and wake time daily)
- Create a cool, dark, quiet sleeping environment
- Use a supportive mattress and pillows (memory foam or latex often suits arthritis)
- Avoid screens for 1 hour before bed
- Limit caffeine after 2pm
- Try a warm bath before bed to ease stiffness
- Use pillows between or under knees to support joints during sleep
- Discuss night pain with your GP — taking anti-inflammatory medication in the evening may help

4. Energy Conservation

Occupational therapists call this "working smarter, not harder." Sit rather than stand for tasks when possible. Use labour-saving devices (electric tin openers, lightweight vacuum cleaners, online shopping delivery). Organise your home so frequently used items are at waist height. Use a trolley to transport items around the house. Batch activities (e.g., cook double portions and freeze half). Ask for help without guilt — energy is a limited resource that needs careful allocation.

5. Nutrition

Diet affects energy levels significantly. Eat regular, balanced meals to maintain stable blood sugar. Include iron-rich foods (lean red meat, spinach, lentils, fortified cereals) if anaemia is a concern. Stay well hydrated — dehydration worsens fatigue. Avoid excess sugar and processed foods that cause energy crashes. The Mediterranean diet supports both joint health and sustained energy levels.

6. Address Mental Health

If you're experiencing low mood, anxiety or persistent distress, seek support. NHS Talking Therapies offers CBT and counselling — self-refer at nhs.uk/talk. Mention that you have a chronic pain condition. Mindfulness-based stress reduction has specific evidence for reducing fatigue in rheumatic diseases. Support groups (Arthritis UK, NRAS) provide connection with others who understand.

7. Discuss Medication with Your Team

If you take methotrexate and experience fatigue, taking it on a Friday evening (so the worst fatigue falls on the weekend) is a common strategy. Folic acid supplementation (usually prescribed with methotrexate) can reduce fatigue. If disease activity is not well controlled, more effective disease management may reduce inflammation-driven fatigue. Iron supplements may help if anaemia is confirmed.

When to Seek Help

Fatigue is often accepted as "just part of arthritis" — but it deserves attention. Speak to your GP or rheumatologist if fatigue is your predominant symptom (it may indicate poorly controlled disease), you experience sudden worsening of fatigue, fatigue is accompanied by other symptoms like unexplained weight loss, fever, or excessive thirst, or fatigue significantly impacts your ability to work, socialise or care for yourself.

Arthritis fatigue is real, significant and manageable. While it may never disappear completely, the strategies above can help you reclaim energy and improve your quality of life. Our virtual assistant can help you develop a personalised energy management plan.

This article is for general information only. Consult your GP or rheumatologist about persistent fatigue, as it may indicate treatable causes like anaemia or poorly controlled disease.

Q&A

Question: Why Does Arthritis Cause Fatigue?

Short answer: Arthritis fatigue is multifactorial — it results from the interaction of several mechanisms:

Question: When should I seek professional advice?

Short answer: If symptoms persist or worsen, pain is severe and unresponsive to usual relief, or you develop new symptoms, contact your GP, physiotherapist or rheumatology team promptly.

Question: Where can I get free arthritis support in the UK?

Short answer: Arthritis UK: 0800 5200 520. NRAS: 0800 298 7650. Free tools and guides at livingwitharthritis.org.uk.

General information only — not medical advice. Consult your GP or specialist before changing medication or starting a new programme. Emergency: 999. NHS advice: 111. Living With Arthritis · livingwitharthritis.org.uk